Confido Health

Software and AI
27th Jan 2026

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Summary

Founder BRCs

Chetan

+ Entrepreneurial
+ "All or nothing attitude"
+ Thoughtful leader; mature for his age
+ IPO scale ambition from day 1
+ Strategic and thoughtful
+ Takes bold steps
+ Resourceful
+ Hard working and dedicated
+ Sales oriented
+ Iterative – work fast fail fast mentality
+ Focussed on team building
+ Admired by the team
+ Followership and referenceability
? Frugal mindset – pay for the best
? Hustle first on sales
? GTM – bad sellers, underconfident (ex-CRO ref - pinch of salt)

Vichar

+ Deep Product Intuition – built strong products at Burncal, Dronation
+ Detailed oriented
+ very outside in
+ Analytical
+ Learning; constantly consuming
+ Complementary to Chetan - Bold + idea guy
? Overanalyses - needs to be more 80/20

Business

Category	Metric	Considerations
Revenue	ARR Growth	• **$4M Contracted ARR Dec'25 (at plan) – grown 4x since May '25** • Live revenue @ $1M; lags by 6 months to CARR • **Proven PMF in MM and ENT** – Closing multiple 100K+ accounts and now closing $500K+ accounts too • Projecting to hit **$15M by Dec'26 (potential to exceed)**

	NRR & churn	• **3 logos churned ($280K CARR); 90% logo retention overall** given they are shedding SMB customers • NRR currently limited since most customers <1 year old
	New Logo Addition	• Strong momentum - added **33 new logos since we met in May'25**; Strong **inbound motion working** • 40% of leads are from perf. marketing; **9K per SQL, 30% conversion, $100K+ ACVs** • **Successfully moved upmarket – Avg ACVs of new logos** expanded from **$63K in Q1 '25** to **$130K in Q4**
Operating efficiency	**GM**	• **30-40%**; **low** given expenditure to Twilio and other infra (Gemini, elevenlabs, etc)
	Key ratios	• **CAC Payback <6 months**; *(CAC per $100K ACV lead is ~$9K)* • **Burning $400K/ month** – High on an absolute terms, but Burn multiple is <1 given CARR added of $500k - $1M. • **$8.2M in bank; 22 months of Runway left**

Customer Chats

1. Sonny, VP innovation @ VNA Health care (Lost deal)
2. Maria, CEO @ Forme
3. Lynett, CEO @ SAMGI
4. Srinivas, COO @ Dallas Renal
5. Ankur Bansal, Founding team@ HCLS (Healthcare services partner)
6. Ruchir Mehta, Co-founder @ Remedo (Dental market platform)

Detailed notes

1. **Sonny, VP innovation @ VNA Health care (Lost deal)**
   They picked Clarion over Confido in a competitive eval and it was a very close call. Overall this was a positive ref call on Confido despite the loss

   - They evaluated Clarion, confido, hello patient + 3 others; Confido and Clarion were selected for 3 week pilots
   - It was a competitive eval and a close call
     - ? Clarion won primarily because they had a better EPIC integrations. Data from Clarion would sync into the EHR faster than confido.
       - E.g., "Clarion typically synced data in a few seconds. Confido was 3-4 minutes in some cases. Our medications needed instant approval on the backend, so this delay didnt work"
     - "Except for the integrations, both these platform were as good as each other"
     - What else differentiated the two?

        - + Confido conversation handling was better. It could triage situations better and handle edge cases. “E.g., Route to emergency if they talk about bleeding"
        - + Voice quality on both were great
        - ? Clarion had better scheduling accuracy in a few cases. “Confido would schedule patients in clinics farther away from their house sometimes"
- On implementation
    - Both Confido and Clarion pitched 90 day go live - a few other vendors were eliminated from the process for pitching 6 months timelines
    - Ultimately Clarion full rollout took 8 months - so this is a feature not a bug in this space
        - "They went live with 5 locations in 3 months. They probably could have gone live with all my 21 locations but I forced them to slow down."
        - Expert pushed for a phased rollout over 5 months - said its normal and nobody wants to rush these things because its customer facing
        - Even tho clarion accuracy was good, they made sure it worked for a month before moving to the next phase of rollouts.

Detailed

Company Background & Use Case (Enhanced)

- Integrated healthcare network (IDN) with 21 locations
    - Mix of primary care, specialty clinics (oncology, rheumatology, behavioral health), home care, and 6 hospitals
    - Operates 24/7, 7 days/week (no downtime)
- Annual scale:
    - 397,000 unique patients
    - ~900,000 total encounters/year
- Call volume:
    - 45,000–47,000 calls/month (stable over last year, expected to rise with new locations)
    - Calls include prescription refills (including controlled/narcotic meds), diagnostic test & lab follow-ups, appointment scheduling/rescheduling, and clinical triage (e.g., “do I go to ER?”)
- Legacy call center challenges
    - 225 total staff (175 internal + 75 outsourced overflow)
    - Average hold time 6.5 minutes (often worse during peaks); patients frequently waited 10–15 minutes
    - Outsourcing helped scale but did not solve wait times or quality consistency

Evaluation Process & Criteria (Enhanced)

- Trigger: exposure to AI call center vendors at a late-2024 healthcare conference; first time evaluating AI call automation
- RFI → RFP timeline: ~4 months total (early 2025)
- Vendors considered: Clarion, Confido, Parakeet Health, Hello Patient, Puppeteer Bridge
- Core evaluation criteria:
    1. Multi-specialty capability (clinics + hospitals + home care)
    2. Epic EMR integration (read + real-time write-back/alerts)
    3. Experience with mixed payer types (commercial, Medicare, Medicare Advantage, Medicaid)
    4. Hospital-grade triage (ER diversion, escalation logic)

    5. Capacity for 50k+ calls/month
    6. Implementation speed (target ≤90 days to initial go-live)
- Early eliminations:
    - Hello Patient: small primary care focus; not hospital-ready
    - Others: lacked true real-time EMR integration (separate portals / batch-like delays)
- Finalists: Clarion, Confido, Parakeet Health
- Parakeet eliminated due to 5-month implementation timeline (vs ~90 days for Clarion/Confido)

Demo / Pilot Design & Results (Enhanced)

- 3-week live demo
- 21 locations split evenly → 7 locations per vendor
- Each vendor handled ~15,500 calls
- Care mix parity enforced: equal distribution of specialties, 2 hospitals per vendor, and home care population
- Tested both inbound calls and outbound calls (appointment reminders, lab/result follow-ups)
- Metrics evaluated:
    1. Response time: all effectively equal; near-instant pickup
    2. Scheduling accuracy: correct provider, specialty, location (proximity-aware), date & time
    3. Triage accuracy: correct pathway (schedule visit vs same-day vs ER vs 911)
    4. EMR integration: speed/reliability of read + write-back and clinician alerting
- Clarion strengths:
    - Scheduling accuracy ~99%
    - Better proximity-based location matching
    - True real-time Epic integration; fewer downstream patient issues from incorrect times/locations
- Confido strengths:
    - Stronger on complex, multi-issue triage (e.g., chest pain + missed med refill)
    - Faster problem-solving across multiple threads
    - ~9/10 script adherence for standard scenarios
- Confido weaknesses:
    - Scheduling accuracy ~93–93.5%
    - Occasional errors on time slots, provider calendars, and location distance
- Confido EMR lag observed:
    - Typical: 3–10 minutes
    - Edge cases: up to ~24 hours (e.g., “need to talk to my doctor” messages not surfacing until next day)

Implementation & Rollout (Enhanced)

- Decision: chose Clarion primarily due to Epic real-time integration reliability
- Rollout strategy: intentional soft rollout; initial go-live within 90 days
- Started with 5 busiest locations (highest call complexity)
- Phased expansion:
    - Month 0–3: 5 locations live
    - Next ~2 months: +5 locations
    - Next ~2 months: +7 locations
    - Final batch: remaining locations
- Total rollout: just under 8 months for all 21 locations
- Operational prep: SOPs and clinical pathways provided to all vendors; internal “super users” trained during phased launch

Operational Impact & Outcomes (Enhanced)

- Wait time: 6.5 minutes → ~42 seconds
- Staffing: ~225 → ~20, with redeployment (no layoffs) of ~130 to other departments
- Quality & safety: calls recorded; QI team performs random audits; no material misinformation/quality issues identified
- Patient experience: significant satisfaction lift; faster resolution and fewer scheduling errors

Pricing & Contract Terms (Enhanced)

- Contract: 3-year flat-fee agreement
- Pricing: ~$1.7–$2.2M per year
- Structure: avoided per-call and per-seat pricing; fee based on known call volume, location count, specialty mix, and hospitals + home care complexity
- Rationale: short-term commitment in a fast-evolving market; maintain leverage if performance slips

Switching Criteria / Risk Factors (Enhanced)

Would reconsider Clarion if:

- Hold times increase
- Scheduling accuracy declines (wrong provider/time/location)
- EMR integration becomes unreliable or inaccurate (delayed/missing write-back/alerts)

  Why EMR real-time matters:
- Medication refills need immediate provider alerts
- Triage outcomes (ER diversion/911) must appear instantly in Epic for proactive follow-up
- Delays can directly impact clinical outcomes and liability

2. **Maria, CEO @ Forme**

Summary

- + Strong feedback and promoter – "Blown away by these guys","It's just crazy how well it work", etc
- + This was a customer call that she took while on vacation "Im on vacation but still taking this.. these guys have been that helpful"
- + 70% calls handled by AI. Had a dedicated 2 person team and now fired one of them; Stopped paying for an afterhour calling service too
- + Rollout was phased over 4-5 months. She wanted it phased by workflows, manually reviewed by her scheduling agents, and then approves go live.
- + "Incredible team, smart and visionary"
- + WOM is strong – she had done a few refs on them before buying.
- ? Few errors on accuracy – " a couple of times it double booked people, but as we reviewed these mistake, the tool really got better"

Detailed notes

1) Overall she is extremely positive on Confido and strong promoter. She had taken time out on her vacation to give a customer this ref call. Chetan had this recorded for us.

Positive drivers:

- Patient acceptance exceeded expectations for Latino, bilingual, older, and immigrant patients.

    - "We have accomplished what I never thought it was actually going to be possible with my population."
- High automation coverage: ~70% inbound handled by AI; ~60–70% confirmations.
- Consistency + fewer errors (DOB/phone validation; standardized rule enforcement).
- Clear operational ROI (FTE reduction, answering service eliminated, fewer unnecessary urgent calls).
- Team quality (execution + responsiveness + transparency). "This is an incredible team… smart… visionaries."

Negative drivers / frictions mentioned:

- Occasional operational errors primarily around double-booking.
- Deep insurance workflows (RTE / Part A/B detection) not yet live for her use case.
- Dialect switching is possible but guardrailed (less dynamic adaptation by design).
- AI documentation/coding not provided by Confido (she uses Freed).

2) Implementation timeline (how long it took)

- She began working with Confido ~4–5 months ago.
- Rollout approach was phased:
    - Started with appointment confirmations.
    - Expanded to reschedule/cancel flows.
    - Then moved to inbound answering (Spanish + English) and broader scheduling.
    - ~3 weeks before the call: AI trained to book new + follow-up appointments and learn insurance acceptance + visit/provider rules.
    - This week: outbound care-gap outreach launched (wellness, colonoscopy, annual GYN), enabled after scheduling flows were learned.

3) ROI / benefits they're seeing

Labor / staffing impact

- ~70% calls resolved without humans; ~60–70% confirmations automated.
- 1 call-center FTE eliminated, 1 repurposed to results-calling; more reductions planned. "There's two people… one downsize and… the other one is… calling our patients with results."

Vendor cost savings

- After-hours answering service discontinued entirely.
- Fewer unnecessary urgent calls and fewer "button-press misroutes" compared to the prior system.

Revenue and quality upside

- Outbound campaigns to close care gaps described as revenue-positive now that they're paid for care-gap performance.
- More predictable operations and better data quality (phone/DOB confirmation).

Workflow efficiency

- AI generates structured telephone encounters in ECW for refills, messages, routing to MAs.
- Standardization reduces human error from inconsistent rules and manual mistakes.

4) Evaluation + pilot process (did they compare Confido to others?)
How she found Confido
- Introduced via her billing company contact while discussing AI.

Selection process
- She interviewed the Confido team (mentions Vichar, Nabil).
- Strong trust signal:"I know Ankar and Ankar highly recommended them."
- She also got a reference and stopped the search early. "I felt I didn't have to… go further… and then we got a reference."

Pilot / rollout mechanics
- Weekly, then biweekly iteration meetings with explicit goals; she says they hit every goal.
- Initially, 100% of AI-booked appointments were manually reviewed via email notifications to a call center rep.

Competitor comparison
- No formal bake-off described; she referenced having had "another platform" previously and replacing the after-hours answering service.

5) Where Confido could be better (gaps / limitations)
Insurance depth
- Her deployment does not currently include deep insurance automation like RTE validation and Medicare Part A/B detection.

Scheduling edge cases
- Remaining mistakes are "very close" to minimal, but issues can still happen around double-booking.

Dialect adaptation
- System can adapt dialect/accents, but they intentionally guardrail it to keep a calm, consistent tone.

Adjacent AI workflows not included
- Provider documentation/coding is not provided by Confido in this setup; she uses Freed.

6) Overall sentiment and word of mouth
- Sentiment: extremely enthusiastic.
  - "I'm like freaking blown away with these guys."
  - "The efficiencies… mind-blowing. It truly is mind-blowing."
- Word of mouth / advocacy behavior:
  - "I'm on vacation… I took the time for this meeting."
  - "You're in great hands."

3. Lynette, CEO @ SAMGI

- Lynette Scherer, MD is CEO at Surgical Affiliates Management Group, Inc.
- They operate 74 practices across 16 states in the US, representing a cross-section of urban and rural regions.
- Practices vary widely in cultural and linguistic diversity; accent comprehension was a key requirement for any voice system.
- Practice types and sizes vary:
    - Small clinics: ~400 operations/year, usually staffed with 1 coordinator handling all tasks including phones and rooming.
    - Busy clinics: ~1500 operations/year, usually with 3 coordinators sharing tasks; in reality, phone duties often default to 1 person.
- They run an exclusively emergency-focused practice — nearly 100% of visits are follow-ups post-surgery or trauma care.
- Current administrative setup:
    - Each clinic has coordinators ($23/hour + 19% benefits).
    - Central/national call centre serves as overflow — staffed by 6 regional managers in rotation.
    - Shared scripts and systems across clinics but with significant local variations in how calls are handled.
    - All calls are routed through VoIP systems integrated with EHRs (Practice Fusion or Epic).
    - The central team steps in when local staff are sick, overloaded, or off-duty.

<u>Current product usage</u>

- Confido, an AI voice agent for inbound call handling, was implemented starting January 2025.
- Pilot started with one trusted clinic where failure risk was tolerable; rollout now extended to 4 clinics including:
    - A site in the Southwest, the Denver market, and two other varied clinics.
    - Full nationwide rollout planned for June 2025 across all 74 locations.
- Deployment approach:
    - Confido spent 3 months in discovery mode, listening to local call patterns and customizing the script per clinic.
    - Final rollout required minimal editing thanks to upfront training and tuning.
- Call volumes handled:
    - ~100 calls/day in smaller clinics.
    - ~300 calls/day in larger ones.
    - ~4-minute average call duration (~400 minutes/day = ~$56/day cost per clinic).
- Performance stats:
    - “Well over 90% of calls are managed entirely by the robot” – includes bookings and direction requests.
    - Patients are offered the choice between robot or human — majority choose the AI.
- Cost model:
    - $8/hour bot talk time (pay-as-you-go).
    - Pilot: ~$500/month.

- Estimated ~$1,500–$2,000/month for small clinics; up to $5,000/month for large clinics depending on volume.
- 3x cheaper than human staff: $23/hour + 19% benefits for coordinators.

"They spent time in our clinics listening to how our people answer the phone and respond… they actually spent a good amount of time learning how we do that and what the algorithm ought to be."

Positive feedback and pain points

Positive:

- Very high automation success rate: 90%+ calls completed by the bot.
- Accent comprehension was strong: key for a diverse patient population.
- Cost-effective: ~$56/day vs ~$200/day for human equivalent.
- Integration was frictionless:
  - No new hardware.
  - Phones already connected via VoIP and linked with EHR (Practice Fusion/Epic).
- Positive patient reception:
  - AI voice is preferred even when human is an option.
  - Surprised the CEO, who was initially skeptical of AI in patient interaction.
- Scalable:
  - Can handle simultaneous calls unlike humans who are single-threaded.

Pain points:

- Initial setup was slow: ~3 months for script development and AI tuning.
- Not robust to off-script conversation:
  - Patients going off-algorithm lead to jarring restarts or unclear responses.
  - Lacks human-level conversational flexibility or nuance.
- Limited proactive intelligence:
  - AI doesn't handle unexpected questions well.
  - Still relies on structured flow.

"If the patient goes off script in any way, it gets a little glitchy."

Impact of the product

Business impact:

- Reduced need for new hiring in admin roles — especially useful in tight labor markets.
- Enabled lighter staffing while maintaining call coverage.
- Lower cost of operations; AI is ~3x cheaper than a human equivalent.
- Greater scalability — no staffing constraints with volume spikes.
- Seamless integration with VoIP + EHR setup — no CapEx.
- Improved consistency in patient communication.

"By moving to a system like this, we're able to staff our clinics a little bit lighter with people and still deliver really good service."

Staff impact:

- Alleviated burden on coordinators who disliked phone duties.
- Increased job satisfaction by shifting focus to direct patient care.
- In small clinics, helped cover phone duties during peak clinical times.
- Potential to restructure clinic staffing in the future (6–12 months out).

Next steps

- Planned nationwide rollout to all 74 clinics by June 2025.
- Considering outbound capabilities (e.g., follow-up reminders) in future use cases.
- Recommended Confido to all other practices owned by their PE sponsor group.
    - Some interest shown; adoption pending.
    - One example: a veterinary clinic where high-touch calling could work well.
- Looking for improvements in:
    - Faster time to deployment across new clinics.
    - Better handling of unstructured/off-script conversations.
    - More human-like tone and empathy in conversations.

"If there was a company that could demonstrate fluid response to a patient who was totally off-script… that would be appealing to me."

4. **Srinivas, COO @ Dallas Renal Group**

Context:

- **User's Role**: Srinivas, COO of a medical group called the Dallas Reno Group.
- **Business Focus**: The group manages 12 offices with 40 doctors and approximately 70 staff members, providing care primarily in the field of nephrology (kidney care).
- **AI Implementation**: They have been using Confido to automate patient interaction, especially related to appointment reminders and administrative tasks.

Core Themes of Discussion:

1. The Current Setup and AI Integration

- **Offices**: 12 offices in total, with doctors and staff answering calls across all of them.
- **AI Adoption Timeline**: Started working with Confido in 2023, with implementation spanning several months and a phased rollout.

- **Confido's Role**: Primarily focused on handling **outbound calls** (reminders, appointment confirmations) and **inbound calls** (routing simple questions such as appointment status, billing, fax numbers).
- **Payment for Service**: The cost is based on the number of minutes used.
    - Invoices typically range between **$30k to $40k per month**, equating to an annual cost in the **$360k to $480k** range.

2. Problems Before AI Implementation:

- **Call Volume**: The group handles **40k-50k calls per month**.
- **Call Types**:
    - 40% are about **appointment reminders**.
    - 10-15% are related to **labs** and **prescriptions**.
    - Many calls are **routine inquiries** (fax numbers, billing, office location).
- **Staffing and Call Handling**: Staff are interrupted frequently with calls, hindering their primary duties (rooming patients, preparing charts).

3. Performance Metrics Post-Implementation:

- **AI's Impact**: Confido now handles 13**% of inbound calls** and **approximately 50% of outbound calls** for appointment confirmations, significantly reducing administrative burden.
- **Patient Response**: AI's outbound calls achieve about **60% confirmation rate** (an increase from prior to implementation).
- **AI's Role in Call Routing**: Calls are being **routed to the correct offices** using AI to screen and identify the purpose of the call. There is a goal to handle **50% of inbound calls** by the end of the year.
- **Operational Efficiency**: The system's automation has reduced the need for dedicated call-answering staff, thus decreasing **staff training time** (previously about **3-4 weeks** per new employee).

4. Use of Data and AI Feedback Loop:

- **Sentiment Analysis**: The AI performs sentiment analysis to rate the tone of conversations (scale of 1-5), helping to identify and investigate negative experiences or technical issues (e.g., call disconnections).
- **Continuous Improvement**: Actively monitor call logs, provide feedback, and help fine-tune the system. They also focus on **negative testing** (e.g., testing for leap years or incorrect date formats) to ensure the system's robustness.

5. Staffing and Resource Management:

- **Staff Size**: Out of the **70 staff members**, **6-8 are managers**, rest are **medical assistants**. The group does not have dedicated staff for phone call management, as it is spread across different roles.
- **Attrition and Hiring**: Attrition has been an ongoing issue, with rates as high as **50% during COVID**. Currently, the group is not actively hiring due to the

automation provided by Confido. Staff hiring is lower, as the workload has been reduced, and AI is handling much of the routine calls.

6. Future Plans and Feature Requests:

- **Expansion**: Looking to increase AI handling to **50% of inbound calls** by the end of the year.
- **Additional Features**: They plan to add functionalities like **rescheduling** and **cancellations** into the AI workflow. There's a desire to **integrate more features**, including better call routing and patient-specific questions like **doctor names**.
- **Customization**: The team is actively working with Confido on product improvements, specifically tailoring the AI's behavior for the unique needs of nephrology practices (e.g., lab-dependent appointment scheduling).

7. Switching or Switching Costs:

- **Barriers to Switching**: Srinivas emphasized that switching from Confido to a competitor would require **significant effort**, as it would involve replacing the entire PBX system and retraining staff. However, if another vendor could provide better **features** (specifically for nephrology), the group would be open to considering a switch.
- **Cost Sensitivity**: The group is very price-conscious, as the medical practice operates on thin margins. Confido is seen as a **cost-effective solution**, with Srinivas preferring it over more expensive competitors that he has evaluated in the past.
- **Regulatory Concerns**: There are **regulatory constraints** regarding the use of AI for certain tasks, such as **transition care management** (PCM) and **remote patient monitoring** (RPM), which require certified human staff (e.g., RNs or LPNs).

8. General Impressions of Confido:

- **Overall Satisfaction**: **7-8 out of 10**, citing that it performs well for **basic functions** but still needs fine-tuning and additional features for more complex tasks.
- **Room for Improvement**: To reach a **10/10**, Confido has to handle a larger portion of the **inbound calls** and provide **detailed call reports**.
- **Potential for Growth**: Sees **scalability** as a key advantage, with Confido allowing the group to handle growing call volumes without needing to hire additional staff.

Additional Insights:

- **Medical Practice Dynamics**: The group operates in a **nephrology space**, with a more complex scheduling system compared to primary care. The need to

ensure **patients complete lab tests** before appointments adds additional complexity to their scheduling.

- **Private Equity**: Mentioned working with **private equity groups**, though these groups have not yet shown interest in adopting Confido.
- **Adoption in Larger Health Systems**: Larger health systems are seen as **slower to adopt** AI solutions like Confido due to their size and administrative layers, but believes there is potential for AI automation to disrupt the **call management** landscape in the future.

5. **Ruchir Mehta, Co-founder @ Remedo (Confido Partner)**

Overall positive feedback on confido but not a deep view since he hasn't seen other tools; Validate that it's a land grab and today most of the opportunity is greenfield; but soon expects the space to get commoditised.
+ Confido feedback generally good from customers
+ Remedo and confide have given each other clients
+ partnership motion in healthcare is not uncommon
+ Events GTM motion is quite legit – some companies purely rely on conferences to sell and hit up 50+ in a year; if done well can be a good source of revenue. Unsure how confide is doing here though
+ mostly run into greenfield oppurtunities
? Watchout for commoditisation in this space – need to own a vertical in healthcare and expand into other parts of the communication stack
? Havent evaluated other players – no deeo comparison with comps but claims they'll be more or less in the range on product quality

Detailed notes

GTM Landscape and Market Dynamics

- Dental market is highly fragmented, irrespective of budget or willingness to pay
- DSOs (Dental Service Organizations) comprise 30-40% of the market
    - 1/3 of DSOs are PE-backed; remaining 2/3 behave similarly to individual doctors
- Market remains fragmented even for large DSOs (e.g. 100 locations)
- Requires both top-down and bottom-up sales motions for adoption
- Inherently an SMB market with some enterprise elements
- Geographic concentration in high-density areas (e.g. NY, Chicago, LA)
- Indians overrepresented in US dental/medical community (3x general population ratio)

Effective GTM Strategies

- Content and community play crucial for individual doctors
    - Active Facebook and Reddit communities for dentists

    - Doctor-to-doctor referrals highly effective (70% conversion rate)
- Cold outreach generally ineffective, but some founders report success
- Conferences are legitimate GTM channel
    - Decision-makers present and actively looking
    - Some companies attend up to 50 conferences per year
- Partnerships are honored and effective
    - Dental CPAs, PMS providers (e.g. Henry Schein, Patterson)
    - IT companies (for on-premise systems)
    - Consultants, fractional executives
    - Real estate brokers and transition specialists

AI and Technology Adoption (+ Confido Views)

- Communication stack evolving from text to voice
- Lots of players in the old world text like weave and podium but they we're only accompaniments to front office
- AI voice is much larger in addressable problem
- AI solutions (e.g. Confido) gaining traction
    - Practices experimenting due to low cost
    - Addressing front office inefficiencies (40-50% missed call rate)
    - Challenges with empathetic handling of patient concerns
- CRM integration for lead management and patient acquisition
- Vertical focus (e.g. dental-specific) may be more effective for startups than horizontal approach
- My notes// (specific on confide not captured well in notes)
    - Good tool and we have taken them to customers (3-4)
    - They have also given us customers
    - Good guys and customer generally happy with the tool
    - Yes there are many other in the market, but they are desi founders "Chamakta hai"
        - Didn't evluate other folks
        - Haven't run into any others while introducing these guys as well
    - Most folk open to experimenting and trying
    - Competitive and fear it will become commoditised
        - Don't see the tools showing major difference from each other honestly
        - The big difference in my opinion is which speciality you chose to support
        - Others are being horizontal but Confido is focussing on dental (confide is also horizontal but he thinks they're dental heavy)
        - Need to specialise and show as many logos as you can
        - That's what simplifies GTM into each segment
    - In the long term need to expand and take over the other tools
        - Like ringer, patient engagement app, etc
        - Cant touch Dentrix, but you can go after other tools in communication stack

        - Better you expand into those areas, otherwise they will build voice AI and start replacing you

Buyer Personas and Decision-Making

- Varies across practice sizes and DSO structures
- Individual doctors heavily influenced by peer recommendations
- DSOs may have more formal procurement processes
- C-suite accessible via LinkedIn for larger DSOs
- Front office staff often gatekeepers for individual practices
- Practice transitions (buying/selling) create opportunities for tech adoption

Unique US Market Characteristics

- Practice ownership frequently changes hands
- Specialized ecosystem of service providers (e.g. dental-specific CPAs, IT)
- High willingness to partner and stay in defined lanes
- Strong community and content engagement among practitioners

6. **Ankur Bansal @ HCLS (Confido's larget partner)**

- Has been engaged with confido for 2-3 years now
- Runs RCM himself and has been a co-founder (CSO) at health-tech co in past
- Implementing them at multiple practices that he manages
- Believes contact centre is a great wedge and team is going to do a great land grab on that
- Big ambition needs to be to get to case management after entrenching with practices
- Believes confido team to have the right intrinsics to do that: understand that healthcare can't be just about selling tech and ROI and they go the mile to build GTM with right narrative
- Delivery post sales is good but they are constrained on tech bandwidth
- Would you invest 1/10th of you private corpus behind this ? 100% yes. NO doubt they'll win the contact centre piece. Journey to deepen from there also confident they can do with adding domain folks
- As per him : founders recognise and have high intent on brining folks from domain which is usually the challenge for tech first cos.

Founder BRCs

- **Manav @ Memora** (Angel and advisor)
- **Mayank Singh** (Tech member @ Dronation)
- **Gaurav Lakhwani** (Angel/Advisor @ Enercomp)
- **Jatin Patel** (Director @ Enercomp Solutions)

- **Mit Mehta** (Founding member @ Burncal)
- **Anik Agarwal** (ex-Cofounder @ Burncal, Confido)
- **Kunal** (Ex-CTO @ Confido, also Co-founder/CTO @ Burncal)
- **Will Nevers** (Fractional CRO @ Confido)

Detailed notes

1. **Manav @ Memora**

   Context: They know each other well and he is an angel and advisor. He's been to Confido's offsite as well. Manav also knows the competitors founders – Assort and Hello Patient.

   - **Big vote of confidence! "If I had to bet on the three founders to win this race - I would bet on Chetan"**
     - **Jon of Assort Health is the more outward facing CEO - he isnt focussed on building a scalable org and it might fall apart at some point. Alex of Hello patient is more in the middle**
     - **Chetan stands out and good balance of both - incredible hustle on sales, and is also building an org that will get him to $20M and past.**
   - + Confident of Chetan hitting $20M
   - + Bullish on Healthcare Voice AI as a space - a lot happens on voice in healthcare - real need. And its one of those spaces that the EMRs dont see as an immediate threat so wont focus much resources on it. Unlike what they did for the scribes.
   - +/? Commodity product and thats okay - "honestly between confido, assort and HP - I dont see a difference"; distribution wins in this space and it's always been the case for healthcare. Even sub par product with great distribution can win (all relationship driven sales)
   - +/? Live ARR to CARR is normal - Most companies have it and chetan has the same problem. Just need to package this story well when he;s fundraising.
     - He didnt have much view on how they're running delivery or onboarding
   - ? What he needs to work on
     - ? Get polished on GTM. As he sells to more enterprises, he needs to learn the art of wining and dining these buyers and also come off as a much larger company. More important for the $20-100M journey
     - ? More short term thinking vs long term at the moment - "I dont blame him for it, it's how I was too and you evolve". But he's very focussed on immediate goals and milestones. Once a while you need to step back and think of the broader vision because his market is evolving very fast
     - ? Ruthless prioritisation. Need to take a few concentrated bets across product roadmap, GTM, etc. Currently he's great at outworking competition, but just this wont scale. Starting to see early signs of this - he's saying no to smaller customers, etc.

   Detailed notes

   Healthcare Voice AI Market Assessment

   - Generally bullish on voice applications in healthcare sector
     - Administrative workflows around patient visit scheduling
     - Phone calls to pharmaceutical companies
     - Voice assistance when patients actively seeking care
   - Market size substantial enough that distribution leaders will dominate

    - Product quality secondary to distribution capability
    - Commodity status less important than market positioning
    - Big players likely to acquire successful smaller companies

Competitive Landscape Analysis

- Knows Chetan very well - no meaningful technical differentiation between players
- Administrative side presents unique challenges
    - EMR companies pose significant risk
    - EMR vendors view scribes as direct threat to their business model
    - Voice agents not perceived as direct threat by EMRs yet
    - No massive comparable companies currently threatening space
- Comparison with competitors Assort and Hello Patient
    - Would bet on Chetan winning the competitive race
    - Knows John (Assort founder) well
    - Chetan superior at internal organization building vs John's external/marketing focus
    - Alex positioned between John and Chetan's capabilities

Chetan's Growth Strategy & Recommendations

- Live ARR and CARR metrics
    - Won't impact fundraising ability significantly
    - Needs compelling narrative for investors
    - Should focus operationally on live deployment delays
    - Shouldn't over-prioritize going live at expense of growth activities
- Founder assessment - very positive
    - Exceptional learning speed and hustle capability
    - Confident path to $20M ARR through pure execution
    - Strong internal team building skills
- Enterprise market requirements
    - Must polish company presentation and personal presence
    - Cannot appear as small player to enterprise accounts
    - Need to master relationship-building game
    - Should become "one of the boys" executives want to socialize with
    - Critical for $20M-100M GTM scaling needs
- Strategic development areas

    1. Long-term market thinking
        - Strong on product long-term vision
        - Needs improvement on market evolution planning
    2. Ruthless prioritization discipline
        - Must learn systematic bet selection process
        - Product roadmap focus vs breadth decisions
        - GTM channel concentration required
        - Currently relies on outworking competition, approach won't scale

2. **Mayank Singh (Tech member @ Dronation)**

- Context: Was an early member and hired for tech expertise
- Engaged mostly with Vichar and Chetan and not Kunal
- Positive & table thumping on the team's ability to cut noise and focus on the solution
- Considers Dronation solution to be class apart
- *"The team was very mature for their age"*

3. **Gaurav Lakhwani (Angel/Advisor @ Enercomp)**

- Context: Investor in Enercomp
- The team was hired for tech expertise and that's what they were doing well
- Wanted a larger outcome or maybe a quicker exit and kept pushing the Enercomp team to look at exits or new geos
- They didn't aling on this growth which is when they peacefully exited

4. **Jatin Patel (Director at Enercomp Solutions)**

Context: They invested/partnered with Chetan and Vichar during their Dronation stint. Enercomp was originally an IT consulting firm working with government agencies and they narrowed their focus to drones/defence only after partnering with Chetan & Dronation.

Net: Table thumping on the founders – hardworking, iterative, great on product and tech. His only call out was that the founders operate with a "work fast and fail fast" mentality which didn't suit Enercomp and hence they parted ways. Given they are more attuned to the gov selling mindset this was a negative for them

- Met them via an employee
- Getting into drone tech was their idea and we are executing it now – Enercomp does 5-6 cr PAT yearly
- It was less of an acquisition but more a barter – we invest and bring contracts and they build the tech /application
- Mature for their age and we are still in touch regularly but less after they moved to US
- **Very hardworking and dedicated ;** Were evolving very fast and realized that drone space wasn't very mature in India
- Got onto the health-tech (Burncal) idea after spending 2 years with us.
- Our tiff was they wanted to raise more money and build a larger team but we want to build an IPO scale profitable company
- Chetan is a great seller – thoughtful, build deep relationships which pays off well for him
- Vichar is the product brain and thats very complimentary for them
- I don't know much about how their US journey has been

- Would have invested in the team but I don't understand the private market well – "raising 40M dollars and burning 40M dollars is not for us"
- They wanted to " Work fast and fail fast" used to keep repeating this to us.

5. **Mit Mehta (Founding member @ Burncal)**

Context: got connected during Bruncal journey via an angel in Burncal; Sanjiv Suri. Started as a consultant but moved to full time after 2 months and also ended up investing. My read is that he is a friend and young person - couldnt highlight any real gaps to double down on

On Chetan

- Even though he is young he leads the team well
- Extremely thoughtful in hiring and being strategic about actions
- We cut burn till we found PMF even though the product was top notch
- He is out and out process and people person
- He is 9/10 for me in terms of founder
- Would join again and invest again if things align
- ? Need to figure how he delegates, large team building is untested

On Vichar

- Extremely good/spikey in product
- He is constantly consuming knowledge to be better at his job
- During Burncal - Built impressive product suite for both coaching side and frontend users
- Very detail-oriented and reference-driven in approach; isnt afraid to ask questions
- ?Should keep the decision making to himself and not crowd with tech leads etc

Between the two of them they have agility and product/execution depth needed to build. Shouldnt have hired the CTO in Burncal / Kunal - he is also a CTO in Confido who may step down soon

6. **Anik Agarwal (ex-Co founder @ Confido)**
Overall: Very positive view on the founders. He has a lot of respect for Chetan and Vichar and holds them in high regard.
    - Chetan's super power – he's bold and goes all in. At burncal he became a fitness trainer himself, at confido he just got on a flight and got his O1. Man of action and committed
    - If he says he'll do something you can rest assured its getting done

- Vichar is the outside in product guy. Always consuming information, always looking at data and always coming up with ideas
- I am older than both of them and had kids – I couldn't move to the US and realised ill be useless if I stayed in India working remote.
- I would join them for sure if I was younger; Angel invest yes – if I had the money, It's a safe place to invest.

- **Background and Early Connection**
  - Met Ketan and Vichar through Anik's brother, who studied at BITS with them.
  - Bond deepened during a startup accelerator program where they all participated.
  - Anik exited his previous startup in 2020 and was looking to start something new.
  - Ketan and Vichar had just exited their company, Dronation, around the same time.
  - The group connected over shared thought processes and values.
  - They built a friendship-first dynamic before becoming co-founders.
- **Founding Team Members and Strengths**
  - **Ketan (CEO)**
  - Chosen as CEO due to his charisma and strong communication skills.
  - Known for:
    - 100% commitment and hustle.
    - Willingness to go all in — a "100 or nothing" mindset.
    - Took bold steps, such as becoming a certified fitness trainer during Burncal.
    - First to get the O1 visa when the company decided to focus on the US healthcare market.
  - Handles customer meetings and sales, though not traditionally trained in structured B2B sales.
  - Admired for being resourceful, committed, and action-oriented.
- **Vichar (Product Lead)**
  - Deeply analytical and data-driven.
  - Known for:
    - Reading deeply to become a domain expert quickly.
    - Driving product direction with a customer-backward approach.
    - Acting as the bridge between marketing and tech.
    - Constantly researching competitors and industry trends.
    - Participating in external learning (e.g., STOA School) to improve his product thinking.
  - Not hands-on with coding, but strong in product vision and prioritization.
  - Encouraged data-backed decision-making across teams.
- **Kunal (CTO)**
  - Anik's childhood friend from Ahmedabad.
  - IIT Bombay Computer Science graduate.
  - Was working in Bangalore for a Silicon Valley startup before a personal accident.

    - Returned to Ahmedabad in early 2020, recovered, and joined the founding team.
    - Technically strong and dependable.
- **Anik (Founding Revenue Role)**
    - Took the lead on early sales and customer meetings alongside Ketan.
    - Played a key role in landing the first customer.
    - Exited after 8–10 months due to not wanting to relocate to the US.
    - Cited family priorities (having a young child) as the reason for stepping away.
    - Believed he couldn't do justice to the US-focused role from India.
- **Reflections on Team Capability**
    - Ketan and Vichar were described as highly adaptable and resilient.
    - Demonstrated ability to transition from a fast-paced consumer app (Burncal) to a slower-moving healthcare market.
    - Learned on the fly how to pitch to doctors and sell into healthcare.
    - Lacked network and connections in the beginning — needed someone to open doors.
    - Grew in patience and maturity as they adjusted to the enterprise sales cycle.
- **Hiring and Team Building**
    - Showed willingness to bet on senior talent early:
        - Hired Mahesh Arangil (30+ years as a certified coach) as head of fitness.
        - Brought on Mit Mehta, a veteran with deep fitness industry ties.
    - Created a credible leadership team across fitness, product, and technology.
- **Personal Reflections and Current Relationship**
    - Anik respects and values the team greatly.
    - Would have continued if younger and without relocation constraints.
    - Described himself as a well-wisher and close friend to the founders.
    - Would angel invest in the company if he had the capital.

7. **Kunal (ex CTO @ Confido)**

Adding a ref from Kunal ex-CTO – he officially resigned this Sunday. We knew he is moving out via Aman but Chetan hadn't confirmed to us hence wanted to wait.

He was the Cofo/CTO at Burncal. Reason to move out: from the beginning I was aware that I won't be able to do this for long since I have family priorities hence I didn't assume the founder role. Still advising them with tech. Aman of Greylabs also validated this reason to leave in our conversation

On Confido:

- We have finally figured out a "solid product and a good narrative"
- Its hasn't been an easy journey of building this product. We are a team of techies and this hospital ecosystem isn't something we understood at all

- It has taken us a lot of beatings to get to this stage of product fit
- *"The company is almost like my child, I have responsibilities towards family business hence I cant continue "*
- Readiness of the product? The product was built with scale in mind from day one. We have gone through multiple iterations to get here
- Team culture? The team has a lot of context with each other and we have worked quite closely – no hierarchy, very frank and easy to work with
- What are a couple of things that people would not like/doesn't work?
    - Payscale! we have operated frugally for a very long time – it gets to the team sometimes. We operate in resource constraint environments only
    - Ended up making a couple of bad hires because of this resource constraint view – which costed us more time issues

On founders:

- Whats their ambition? Build a big company from day 1 and get to IPO
    - Will they sell if Assort offers to buy this?
        - We got an offer for 51% when we started which we rejected
        - Very difficult to comment at the moment as we don't know if it's a 300M val/500?1b?
- Would you join again?
    - I am going to my family business but if things were not this I wouldn't have left
- Advice to the team?
    - Build a strong team
    - Tech is evolving very very fast for the space they are building in. We need to keep up and evolve everyday
- On Chetan
    - "Chetan really excellent in what he does"
    - He is thoughtful and looks at the bigger picture more
    - On leadership front he is good and balances firm and open
    - "there is nothing I can point towards as a flaw for Chetan. He is very good"
- On Vichar
    - He is the bold one amongst the two and has more ideas
    - He would do better if he doesn't over analyse the problem too
    - Sometimes its not helpful at all to go to this depth and we can take a quicker decision route. He is a 100% invest yourself 80/20 approach
- Would you invest in this team – like 1/10 of your investing money?
    - I am investing both my time and money as much as I can

8. **Will Nevers, Fractional CRO @ Confido (was fired)**

Overall Negative ref (pinch of salt since he was fired and they are on bad terms)
Team had no scalable GTM, founders couldn't sell, and weak product vision. Would agree to building anything a prospect would ask for and never said no.

Confido Experience

- Contractual role as CRO to build GTM and pricing structure
- Found company was very early stage:
  - Customers lacked traction
  - Difficult to build case studies/success stories
- Pricing challenges:
  - No established pricing model
  - Debate between usage-based (like newer AI models) vs. subscription
  - Couldn't get commitment on pricing direction
- Attempted to set up sales process:
  - Implement CRM and pipeline
  - Develop better messaging
- Company made promises first, tried to build later
- Logistical issues:
  - Company not based in NY; founders primarily in India
  - Difficult to schedule meetings with founders
- Founders wanted more of a lead gen person than strategic CRO

Product and Competition

- Confido vs. competitors:
  - Lindy AI seen as more advanced
  - Confido required manual setup of conversation flows
  - Competitors used AI to automate conversation flow building
  - Confido less automated in infrastructure and setup
  - Cumbersome to provide value and integrate with practices
- Other companies had better infrastructure, workflow, and product clarity

Sales Challenges

- Founders lacked confidence in their own product
- Conference experience:
  - No booth or presence prepared
  - Only AI company present
  - Founders struggled to simplify and sell effectively
  - Came across as timid about the product
- Handed over customer conversations to me on first day
- Lack of urgency in closing sales
- Founders okay with losing potential customers
- Difficulty answering in-depth questions

Product Metrics and Integration

- Attempted to implement product metrics:
  - Time saved, efficiency improvements
  - Build out pricing based on demonstrated value
- Customers unclear on product value
- Low product stickiness
- Proposed EMR integration:
  - Partnerships with Athena, other tech-forward EMRs
  - Suggested revenue share model with EMRs
- These integrations not implemented during my tenure

Sales Pipeline and Market Focus

- Overly optimistic about pipeline:
  - Added interested parties without formal agreements
  - Assumed interest equated to deals
- Focused on dental groups:
  - Identified as untapped market for AI
  - Targeted marketing efforts towards dental practices

Sales Process Issues

- Easy to get introductions
- Conversations often derailed into technical discussions
- Feature-focused rather than value proposition-centric
- Agreed to build custom features for potential sales
- Used recorded demos instead of live product demonstrations
- Prospects interested but not fully convinced after founder interactions

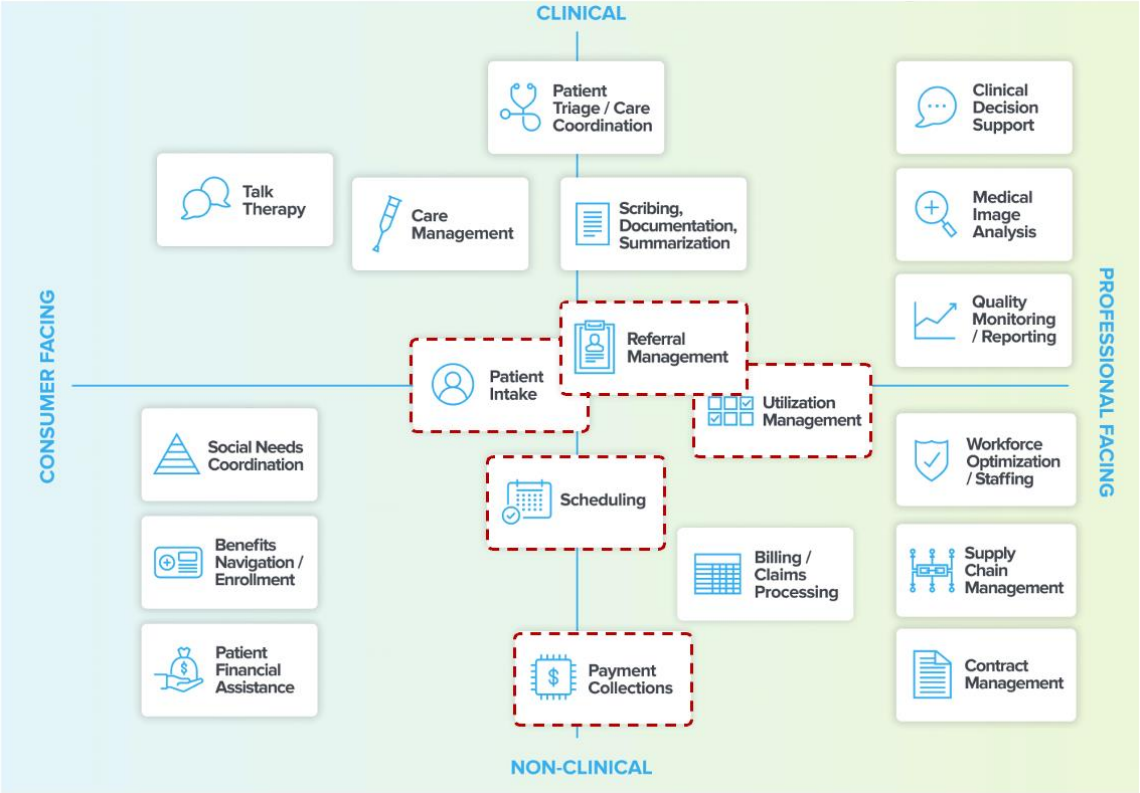
Competitive Landscape

- Some leads were piloting competitor products
- Struggled to differentiate Confido from competitors
- Suggested focus areas not implemented:
  - Reimbursement automation
  - Collections improvement
  - Tangible financial benefits not demonstrated

Heathcare Voice AI market

Jobs to be done

- While AI front doors include a variety of jobs to be done (referral management, patient intake, scheduling, etc.), Confido today primarily focuses on:
  - Outbound calls (reminders, appointment confirmations), and
  - Inbound calls (routing simple questions such as appointment status, billing, fax numbers)
- A typical mid-market specialist group (10+ locations, 40+ doctors, $10-50M revenue) would handle **40k-50k calls / month**
  - 40% are about appointment reminders.
  - 10-15% are related to labs and prescriptions.
  - Many calls are routine inquiries (fax numbers, billing, office location)
- AI solutions here help **route the call** to the correct office, **screen and identify the purpose** of the call thus **reducing the need of dedicated call answering staff**



Typical tech stack

Typical administrative tech stack for healthcare or dental practices in the US range across 5 types of systems. Patient engagement is amongst the least in terms of importance and spend but is medium to large practices and still spend $100k+ on these systems (>10-12 locations)

Table below is a 101 on the tech stack via a specific example of Shared Practice Group – Large DSO in Arizona. (Listed in order of criticality to the function)

Typical Tool stack		**Example from Arizona's largest DSO**	
Tool Type	**Purpose**	**Key Vendors**	**Commentary & Market Insights**
Practice Management Software (PMS)/ Electronic health record (EHRs)	Stores patient records, clinical notes, appointments, contact info	Open Dental, Dentrix, -dental; Athena for hopsitals	**These tools are essential for any DSO**, **3 players dominate** 90% of the market. **Open Dental**, **Dentrix, Eaglesoft**
Performance Management Solutions	Leverages PMS data to drive business efficiency and performance	Dental Intelligence, Peerlogic, Patient Prism	**Typically consolidated with the PMS category now**.
Revenue Cycle Management (RCM)	Handles billing, insurance claims, and payments	Dental Intelligence *(SPG didn't do a deep eval)*	**Dental Intelligence** expanded into RCM along with other tools, offering a broader all-in-one solution.

Patient Engagement Solutions	Text messaging, email reminders, online appointment scheduling	PBN, Dental Intelligence, Weave, NexHealth	Commoditised space. **Players like PBN** and **Dental Intelligence** provide broad patient engagement features, and multiple smaller players too. E.g., Emitrr
Call Analytics	Provides insights into call data to improve phone handling and training	Peerlogic, Patient Prism, RingCentral (RingSense)	**SSG group uses Peerlogic** for its quality and user experience, but will consider **RingCentral's AI** if its improves. This will help them consolidate all into Ringcentral.
Conversational AI/ AI call centre **(Confido current category)**	Automates patient interaction (e.g., appointment scheduling, queries)	Assort Health, Hyro, Penciled (Penciled In)	**Assort Health** was evaluated and is being used in some clinic. Currently thin stack and part of the same budget for patient engagement and call analytics

*In red above is the typical spend allocated to call centre and patient engagement function.*Listed above in red is the typical budget towards patient engagement and call centre – category that confido plays.*

Almost all sizeable businesses have a patient engagement and call analytics offering today. However each of the companies in these categories are not $Bn revenue companies because of fragmentation.
Source: Expert chat – VP of strategy at Shared practices group

<u>Why now?</u>

- **Pre – AI speech models for healthcare (2023)**
  - Before companies like Confido, patient-facing voice use cases were managed through **Interactive Voice Response (IVR)** systems and **call centers**
  - These systems offered **pre-programmed phone tree** to schedule, reschedule, or cancel appointments. Key players included:
    - **NICE inContact:** horizontal leader in cloud-based contact center, healthcare administrative modules offered as well
    - **Genesys:** CX platforms with IVRs integrated into call center solutions, often used by healthcare providers to streamline appointment scheduling and customer service
    - **Avaya:** Legacy player; supported providers with both inbound and outbound calling automation solutions
    - **Aspect Software:** IVR and call center automation services, including patient appointment reminders and outbound calls
  - Key limitations were rigid scripts and poor interpretation leading to fall off in customer adoption
    - Healthcare providers also relied heavily on human operators, which introduced inefficiencies and high costs
- **Post AI speech models getting advanced**
  - Surge in demand/adoption largely due to advancements in accuracy, context understanding and ability to handle healthcare terminologies

- **Voice models:**
  - Companies like **Deepgram (Nova-2, Sept '23), Assembly AI (PII/HIPAA compliant in Dec '23)**, , and **Cartesia** developed healthcare specific speech recognition models – better handling of medical lingo, understanding patient intent and providing more nuanced responses
  - AI systems like **Google Cloud Healthcare API** and **Amazon Transcribe Medical** offer tailored solutions for clinical environments

- Detailed context on evolution of digital patient engagement solutions from text -> voice based and scaling challenges mentioned in appendix

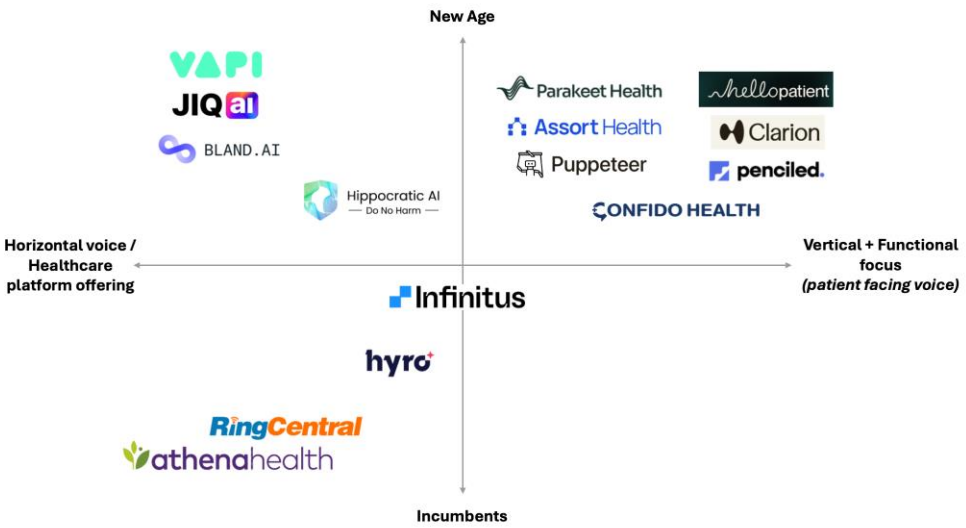
Pain Points Addressed / Value Prop

- Reduced admin burden
  - Reduced need for dedicated staff, decrease in training costs
  - No replacement hiring for attrition – AI filling space
- Net new business from outbound calling
  - Appointment reminders, periodic reminders to schedule appointments (eg. 5 weeks, 10 weeks automated outbound)
- Visibility into "what my patients are saying" – speech call analytics

Competitor landscape

No clear winner yet, but multiple smart money early stage companies building.

- Assort + Hello Patient are the two best-known healthcare-native voice scheduling players (recent large raises). They both compete head on with Confido, and all have similar feature parity.
- There is some competition from adjacent players, but none pose a serious threat to this space yet.
  - Adjacent automation players (Infinitus, Hippocratic, EliseAI) focus on mid/ back office like prior auth, benefits verification, etc. Patient engagement products have lower adoption and are not as mature.
  - Horizontal voice AI (Bland, PolyAI, Jiq, Replicant, Vapi) starting to focus on healthcare- mainly SMBs, weak on EHR depth.
  - EHRs (Epic, athena) experimenting with voice but focused on ambient scribes. Patient engagement hasn't become a focus yet.
- Smart money investments in this space
  - Assort has raised $100M in total. Last round in Aug '25 was led by lightspeed - $50M @ 750M post
  - Hello Patient raised $22.5M in Sep '25 from Scale, 8VC, etc.
- Competitor landscape below:



Competitors funding details and relevance to Confido

Company	Founded	Stage	Raised $M	Last Raised	Investors	Comments/ relevance to confido
Assort Health	2023	Series A	100@ 750M	Aug '25	Lightspeed, Tau Ventures	**Most popular and highest traction** in this space Good product NPS
Hello Patient	2024	Series A	22.5M	Sep '25	A16Z	**Good product and replacing Assort** in a few deals. Similar scale as Confido.
EliseAI	2021	Series D	110	Aug'24	Sapphire, Point72	**Started with real estate call automation**. Now expanding to healthcare. **Products needs maturing**
Hippocratic AI	2022	Series B	278	Jan'25	Premji, GC, a16z, Greycroft, KP	Primarily focussed on mid/ back office workflows like Prio auth. **Front office adoption low**
Hyro	2014	Series B	50	Dec'24	Healthier Capital, Macquarie	**Started as a chat agent** on websites, phone. Good integrations **but weaker adoption** for deep scheduling or triage **w/ voice.**
Infinitus	2019	Series C	103	Oct'24	A16z, KP, Coatue, Google	**Prior-side. Automates payer phone calls** for benefits verification + prior auth. Excellent at high-volume robotic calling; **less applicable to patient**-facing tasks.
Parakeet Health	2023	Seed	3	Oct'24	Canvas Ventures	Patient engagement. Voice agent + call automation. Easy deployment + good UX; **weaker on enterprise-grade customization.**

Arini.ai	2024	Seed	0.5	2024	Y Combinator	Patient engagement **(dental-heavy). Small footprint outside that.**
Actium Health	2014	Acquired	35	May'21	Acq. By Syllable AI in Mar'24	**CRM + outreach engine** for recall, reactivation, and care nudges. marketing automation; not voice agent.
Clarion	2023	Seed	0.5	2024	Y Combinator	**Payer-side.** Focused on claims, auth, and payer-side automation. Good RCM backbone but not dedicated.
Penciled	2022	Seed	0.1	2024	Y Combinator	**Subscale**
Puppeteer	2023	Unfunded	-	-	-	**Prior-side.** Agentic automation for back-office admin

Detailed competitor case studies: https://docs.google.com/spreadsheets/d/1Mg9Fk-FwxOVCdq0u1RrEh2bYP2QGBl_O6fcwC-73l-o/edit?gid=0#gid=0

Competitor Deep dive - Assort

1. **Assort health – Confido's largest competitor**
    - **Currently at ~$10M ARR;** 3 quarters ahead of Confido. **Live ARR at 25%**
        - Trajectory was $0.7 - 9.6M in a year as of August '25.
        - Average ACVs - $130K
        - Live to CARR is 1:4; ~$2.5M live
        - Upmarket motion starting to work: Closed six $500K deals and one $1M CARR deal in the last 2 quarters
        - Sales cycles are 60 days; implementation cycles will range 3-6 months
    - **Live to Contracted cycles are 6 months** – typically 3-4 months for the initial phase to go live and another 3months to rollout across locations.
        - **Standard implementations take 3-4 months**; Most of the time is spent configuring call patterns like you would train a human; Trainings material and SOPs are provided to Assort engineers
        - **The next 3 months are location wise phased rollouts;**
        - E.g., "Assort signed Northern California Retina in Dec '24, built the workflows from Jan-march '25 and went live in April '25 with half the contracted volume of calls."
        - "In another DSO, they tested one location for 60 days, a group region for the next 60 and then 30 days for a full rollout to 36 locations."
        - **In specialities where they have past case studies, implementations are faster and more templatised**. Barington orthpedics went live in **4-5 weeks** because the practice had a good scheduling system in place, and Assort also had templated from their previous deployments.
    - **Assorts ACVs are in the $150K range** for most mid-market specialty groups; **smaller customers range from $30-50K**; Moving from **pure usage based to more predictable models.**
        - Assort **initially priced on per call usage basis**. "You're paying $8 per call, we'll charge you $4 per call" – a DSO

        - They still fundamentally charge on usage, **but pricing model is structured to be flat fee/ minimum guarantee now**.
            - Flat fee. E.g., 15000 calls a month, post which 0.05 per call charged on overage. Overages usually collected quarterly.
            - Per location. $500-700 per location with a fixed bank of calls. And Overage charges if you exceed it.
    - Other metrics
        - Assort typically benchmarks pricing to one US FTE at minimum now. i.e., $5000/ month
        - They typically take on 50% of a providers call volume which is mostly inbound calls for new patient inquiries, scheduling calls, intake calls, etc. Outbound call volume limited to reminders.
    - Multiple use cases pitched by Assort to customers, and **ROI promised can range from 3X to 7X**:
        - **Call overflow/ abandoned call**. Usually 10-15% calls missed during working hours and assort bring this down to 0.
        - **After hour coverage**. Usually 25% of call volume comes after hours and Assort can handle this.
            - Both points above increase patient volume
        - **Scheduling call burden** which can be 30-40% of call volume; Assort handles end to end scheduling. "Free'd up 1.5 FTEs for us"
            - Existing patient calls for payment, confirmation, cancellations. "Replaced $100K outsourced vendor with assort"
        - **Call triage and routing**: "AI handles existing patient calls, and unrecognised numbers get routed to agents. Agents increased focus on high value calls from 20% to 60%"
        - **Followup and reminder calls** (Outbound) are typically expansion use cases for assort, bumping contracts up 10-15%. New call volume.
        - **Staff/ outsourcing reduction**. Some customers have cancelled 20 seat contract centre contracts.

<u>Assort customer feedback</u>

Positives:

1. **Regarded highly for their deep EHR integrations** and compatibility with the rest of the stack.
    - "It has deeper connections with athenahealth and more maturity in that connection compared to competitors like Parakeet"
    - "Assort Health has the **best partnership with Athenahealth**"
2. **Willingness to customise workflows** and specialisation in areas like dental, ortho, etc
    - "Assort was willing to customize and **build out-of-the-box solutions for specific needs**, which was the "biggest deciding factor" for one DSO that competitors refused to do"
3. **Product experience** in the pilots – Ability to navigate conversation and sync with EHR, Good conversation quality, etc
    - These are **more subjective in nature and vary from customer to customer**. Some prefer competitors to Assort as well.

Negatives:

4. **Functional limitations on the conversations – "Sounds like a bot"**, "converting new patients Is low", "Cant handle conversation where patients are confused, broad of looking for guidance"
5. **Features like reporting, payment, etc not great**. "analytics is barebone", "can take payments over the phone"
6. **Pricing** and sales aggression (low priority). "Sales process is aggressive and they don't budge on pricing"
7. **Onboarding** and implementation **experience was negative in the early days**. Assort didn't have sufficient CS bandwidth back then

<u>Reasons for lost deals across competitors:</u>

Most commonly sighted reasons for loosing deals are "unwillingness to customise", "pricing", "pace of implementation/ deployment". This indicates that **product quality is similar across players** and you're **winning on servicing, pricing, integration** in bake offs.

1. **Assort Health**
    - Lost to Flip AI (TrustCare Health): "Aggressive" sales tactics. Flip offered more functionality for the price.
    - Lost to Hello Patient (CortiCare): **Lost** because **Hello Patient offered superior customization** and deeper analytics/reporting. "Assort wasn't willing to build custom requests"
    - Lost to Genesys (Healthtech Company): Lost customer because Assort was "very niche" and intent-focused, **failing on broad, general conversations**.
    - Risk of Loss to Dentina.Ai (Shared Practices Group): Faces pilot threat as **Dentina is 40% cheaper; Assort refused to budge on price**
2. **Hello Patient**
    - Lost to Assort Health (Shared Practices Group): Lost the DSO deal because they were **unwilling to build custom features**; sales pitch was too rigid.
    - Lost to PolyAI (DSO/Pacific Dental Services): Lost during evaluation; was less transparent about the roadmap and was not as analytics-driven as PolyAI.
3. **Elise AI**
    - Lost to Assort Health (Healthtech Company): Lost the client (who later churned from Assort) **primarily because Elise had significant development delays.**
    - Almost Lost to Smaller Vendor (Dermatology Partners): Customer temporarily engaged a smaller vendor when **Elise was "not making progress"** on initial features.
4. **Parakeet Health**
    - Lost to Assort Health (Luminary Hospice): Lost the hospice customer due to Assort's better AI **quality in clinical conversations** and **streamlined EHR integration.**
    - Lost to Assort Health (Prior Customer): Integration was manual, taking 3.5 weeks, compared **to Assort's 10-day quick pilot deployment**.
    - Lost to Hyro/Outbound AI (VNA Health Care): Lost due to **rigid pricing;** demanded a per-user, per-agent fee over a flat fee.

Product requests/ need gaps that customers want:

1. **Revenue Cycle Management (RCM) & Financial Automation**
    - **Payment Collection:** DSO VP - "AI should just take the payment instead of transferring."
    - **Prior Auth & Benefits Verification:** Customers explicitly want solutions that can perform prior auths and benefits verification by **making calls to payers (Blue Cross, Aetna, etc.).** While Hello Patient can already do this, it "stumbles after much more complexity" and defaults back to a human agent for documentation issues
    - **Billing/Claims Resolution:** Leaders call this the "biggest challenge" and ideal for automation – *Utopian ask because payers wont open engage with an automated bot atleast in near term*
2. **Deeper EMR Workflow Integration**
    - **Create EMR Cases from Voicemails:** Customers want the AI to automatically create a patient case in the EHR (like athena's patient case feature) that can be managed by a nurse via text, avoiding phone tag.
    - **Smarter Scheduling Logic:** DSO VP: "If the first slot fails, AI shouldn't dump it back to staff."
3. **Advanced Analytics & Usability**
    - **Caller Sentiment Tracking:** TrustCare COO: "Managing 100 calls means nothing if 90 leave angry."
    - **Built-In A/B Testing:** Telehealth PM: "Need native A/B testing, not vendor-pulled reports."
    - **Full-Funnel Analytics:** DSO VP: "We want analytics on *all* calls, not just AI-handled ones."
4. **Unified Communication Platform (Phone, Fax, Text, Web)**
    - **Outbound Communication Expansion**: Customers want the AI to perform more varied outbound functions such as messaging/ calling regarding low-risk **medical care follow-ups**.
    - **Referral Management (Phone + Fax + Web):** Retina CAO: "Take over the whole referral flow." – *Not a homogenous ask since it sits with separate teams today*

Health system survey (based on expert network responses):

Org	Tools Used / Evaluated	Use Cases	Key Metrics
Northern California Retina Vitreous Associates	• Assort Health • Parakeet Health	• Call center ops • Missed calls reduction • Call volume reduction	• Missed calls • Call volume
University Radiology / Genesis Oncology	• Kogan • Superdial • Infinitus • Assort • Hello Patient • Hippocratic, etc.	• RCM automation • Patient reminders • Clinical notes • Claims management	• Productivity • Staff time freed • AR improvements

Shared Practices Group	• Assort Health • Ring Central	• Missed calls • Phone systems • Rollout planning	• Call volumes • Missed calls
Acellus	• Parakeet • Hello Patient • Clarion	• Clinical documentation • Patient communication • Outcome monitoring	• Engagement rates • Documentation time • Outcome metrics
Pacific Dental Services	• Assort • Hello Patient • Clarion • PolyAi	• Call center ops • Patient scheduling • Insurance workflows • Payment Collections	• Call processing • Patient satisfaction • Conversion rates • Cost savings
UCSF Health	• Assort Health • Clarion • Parakeet • Hello Patient	• Scheduling • Triage automation • Patient intake • EHR cleanup	• Wait times • Error rates • Patient satisfaction
SAMGI	• Confido, AirLEGIT	• Scheduling confirmations • Inbound calls	• Call volume • Human response conversion
Dallas Renal Group	• Confido	• Engagement • Reminders • Scheduling	• Call/text volume • Confirmation rates • Routing accuracy
MercyOne	• AssortHealth	• Orthopedics	• $180k - $200k spend

Voice AI Models and Forecast

- Nuance will go EOL; this made the healthcare segment more attractive for voice model providers – big whitespace
- 3 players in the running – Deepgram, Cartesia, Assembly
- Today providers have a horizontal model (Nove 3 for Deeprgam) which understand names and addresses
    - On top, medical data is finetuned so it can understand symptoms, treatments and medication names.
    - For big clients Deepgram trains on some additional data clients might have
- Can plug in Deepgram and it will be able to understand most of the conversation for admin workflows
    - Founder building on top need to focus on integrating into workflows and systems, and provide logic.
- Scope of innovation at model layer is minimal. Only Hippocratic did a good job but that's because they found a way to get access to data from Nurses (prop data)
- You will need some tweaks to help the model understand a few complex words/ edge cases.
- **Future of voice**
    - S2S is something the LLM providers are bullish on. If you plug an LLM today, it can drive latency up to 1-2 second. To reduce this they needed to come up with speech to speech
    - Nobody is sure of where this will head, but lots of money being poured into it

- As of today it hard to think of S2S for something critical like healthcare because:
    - You need to integrate with underlying systems – most work in text and need function calling
    - S2S is a black box. You give it an input in speech and get an output in the same. You don't know how the model interpreted the questions and how it framed its answer
        - Observability is a big piece that research is trying to solve

Regulatory Landscape

For Confido, **no specific US federal regulations solely targeting AI in patient comms currently:**

- **HIPAA Compliance:** Any AI system handling patient-facing communication in healthcare must comply with HIPAA. Particularly important for appointment management, payment collections, given PHI exposure
    - **Data Privacy:** Robust security features like encryption, access controls, and audit mechanisms to safeguard PHI handled by AI systems
    - **Business Associate Agreements (BAAs):** Confido provides AI solutions to healthcare providers, and will likely be considered a Business Associate under HIPAA
        - Need to enter into BAAs with clients, acknowledging responsibilities for PHI protection
    - **Data Sharing:** Confido transfers data between different entities (e.g., hospitals, payment processors), ensuring the security of this data and managing patient consent is critical

- **FDA Oversight May Apply:** Confido mentions pre/post op engagement which can fall under *"clinical decision-making or providing medical reco"* and hence get classified as Software as a Medical Device (SaMD) by the Food and Drug Administration (FDA).
    - **SaMD Classification:** AI communication tools that assist with clinical decisions, treatment plans, or personalised care recommendations could qualify as SaMD
    - **Evolving Guidelines:** Guidelines still evolving and need to monitor. Currently, AI tools in patient-facing comms fall outside direct FDA purview unless they directly influence clinical care

- **State-Specific Regulations:**
    - **California's AB 3030:** Effective 1 Jan'25, in California, if gen AI is used to create written or verbal communications about patient's clinical info, companies must disclose this to the patient with a disclaimer and provide instructions on how to contact a human healthcare provider
    - **California's CCPA/CPRA:** Stringent requirements for consent management and data privacy. AI tools for appointment scheduling or comms must allow patients to exercise their rights under these acts (e.g., data access, deletion)

- **Other State Laws:** States like Colorado and Utah have transparency requirements when consumers interact with AI systems

- **Transparency, Disclosure and Risk:**
  - **Informing Users:** Patients generally have a right to know when they are interacting with an AI system rather than a human.
  - **Explainability:** While not always mandated, providing explanation of how the AI system operates, particularly for more sensitive communications can be demanded
  - **Assessing Risk:** Need to be watertight around potential risks (e.g., errors in appointment scheduling, incorrect payment information, misunderstood post-operative instructions)

- **Future Regulatory Landscape:**
  - **Proposed Legislation:** Proposed federal laws
    - Better Mental Health Care for Americans Act (positive implication – act explicitly recognises use of AI to:
      - Determine coverage and treatment plans
      - Potentially improving personalized care, and
      - Patinet comms about benefits and treatment options
    - Health Technology Act of 2023 (positive implication –
      - AI may be eligible to prescribe drugs if authorized by the FDA
      - Supports development of regulatory standards for AI technologies in healthcare (transparency, accuracy, patient data privacy)
  - **Focus on Algorithmic Fairness:** Future regulations might increasingly address potential biases in AI systems, particularly concerning diverse patient populations
  - States like Connecticut, Florida, and Illinois are proposing laws that restrict prohibit AI as sole/primary method for adverse healthcare decisions such as claim denials (RCM) or treatment determinations

Competitor pricing models (Assort)

Pricing Model	Customer / Segment	Specific Contract Details & Terms	Annual Contract Value (ACV) Examples
1. Flat Fee / Subscription with Call Volume Cap	Northern California Retina Vitreous Associates Medical Group (Retina Specialty)	Structure: Flat fee/subscription based on capacity. No implementation fee. Capacity/Overage: Handles up to just under 15,000 calls a month. Anything above the cap is charged at $0.05 a call. Term: Includes a 3-month out clause if KPIs are not met. The same price will stay until they scale to 13,000 calls.	Over $50,000 annually. The monthly cost is more than $3,000 or under $5,000. This is negotiated to be the equivalent of one North American staff member per month.
2. Expanded Flat Fee / Unlimited Use	Barrington Orthopedic Specialists (Orthopedics)	Structure: Flat annual price, renegotiated to include unlimited phone calls processed. Initial Price: Was between $60,000 to $70,000 a year for about 2,500–2,600	Low six figures/Around $150,000 a year. The total cost was described as around $150,000

		engaged calls. Renegotiation: Price increased by about $30,000 to add 24/7 call coverage and transcription features. Discount: Received a discount of at least half off what they would normally be charged for the added items, because they served as an early adopter/development partner.	(implied from original price plus the added $30,000).
3. Monthly Fee Per Location (DSO Model)	Shared Practices Group / 4M Dental Implant Center (DSO)	Structure: Monthly fee per office/location with a bank of calls, reconciled quarterly with overage fees. Overage: Initially calculated at $2 per call after exceeding the allotted bank. Terms: Agreed to pay for a full year up front after a successful 90-day pilot.	~$. The negotiated base rate was about \$550 to \$650 per location. This model resulted in the ACV for all 36 locations being comparable to other crucial software (e.g., AI imaging, CRM).
4. High-Volume	Luminary Hospice (Hospice Service)	Structure: Annual spend, implied subscription/capacity model.	$150,000–$200,000 range annually. This translates to a monthly expense of over $12,000. This price covers handling approximately 5,000 calls annually for triage and after-hours coverage.

Competitors implementation cycle and go live

Assort health

Implementation Duration	Customer / Segment	Implementation Phase Details & Involvement	Phasing & Go-Live Status	Customer Feedback on Implementation
Fast (3.5 to 5 weeks)	Barrington Orthopedic (Orthopedics)	Timeline: Signed agreement late September/beginning of October; went live right around Thanksgiving/End of November. Implementation took about three and a half to five weeks. Involvement: The process was "very smooth". The practice had already established a lot of the rules and physician preferences due to a previous online scheduling system, providing Assort with a template to adapt and input into their software.	Go-Live: Went live on all scheduling calls (5,000–6,000 monthly volume). They continued to tweak the system post-go-live. Assort was initially brought in as an extra agent during business hours but quickly expanded to 24/7 coverage.	Highly Positive: Rated the implementation process highly, noting Assort's ability to adapt quickly. The CEO stated the implementation was "very smooth". Assort later made "substantial improvements" to the software itself.
Standard/Complex (~3 months)	Northern California	Timeline: Contract signed mid-December; the designated build-period was January through April.	Phasing: Started with a soft launch in one to two locations. The	Positive: The customer was happy that Assort took the time to "get it

	Retina (Retina Specialty)	They went live on March 28th. The total implementation took about three months. Involvement: This timeline was required because they were Assort's first retina practice. The time was equivalent to the 90 days taken to train a new human staff member. The practice provided training material and algorithms to Assort's engineers, who would program and test the system. EHR Integration: athena integration is "really easy and quick" because Assort is already on athena's Marketplace. The process involved data gathering and input, specifically feeding Assort their SOPs, identifying top 100 payers, and tackling complex diagnosis-based scheduling.	initial goal was to get 2,500 calls off the front desk. The initial deployment focused on scheduling-based calls (about 50% of 10,000 total calls). Status: Assort picked up 3,400 calls in the first full month (April).	right" and not sacrifice quality for quickness. The process helped the customer improve their internal SOPs and workflows. Switching Cost: For other retina practices, the build-out would likely be quicker (e.g., a month and a half), as they can follow the initial template.
Staged/Delayed (~6 months)	Shared Practices Group (DSO)	Timeline: Started onboarding Q2 2024, piloted Q3 2024, and rolled out fully on Q4 2024. The entire process took "a whole quarter" (three months) to go live on the pilot. Involvement: The customer wanted Assort to customize things and build solutions specific to their full-arch implant workflows (e.g., custom bridge to Open Dental and Salesforce). Initial Challenges: Assort was "brand new" and lacked dedicated customer success personnel, making initial onboarding "extremely difficult" and "terrible".	Phasing: Started with a pilot of 10 offices. Initial use was just call overflow management (answering on the fourth ring). Rollout was staged—tested one location for 60 days, a group region for 60 days, and then a 30-day plan rollout across the rest. Status: Fully live on the full system across 36 locations by Q4 2024. This included the custom phone lookup triage.	Negative/Mixed: The onboarding experience "sucked" and was "terrible" initially because they lacked a customer success team, requiring patience from the client. It took way longer than hoped (two and a half months just for the pilot go-live). However, the ability and willingness to build custom features was the "biggest deciding factor" for choosing Assort over competitors.

Hello Patient

Implementation Duration	Customer / Segment	Implementation Phase Details & Involvement	Phasing & Go-Live Status	Customer Feedback on Implementation
Fast (4 to 8 Weeks)	CortiCare / Texas Medical Center (RHC/Hospital Outpatient)	Timeline: Evaluated in Q1 2024 and implemented in Q2 2024. The sales team had pitched a faster implementation timeline compared to Assort Health, which was a factor in the selection. Involvement: Integration is split: Hello Patient does 70% of the work, and the client team does 30%. The IT Director noted that	Deployment: The system was deployed in select service lines, initially Pediatrics and OB-GYN. Time-to-Value: ROI is not expected in the first six months; the organization starts	Positive: Hello Patient scored a 9/10 in the evaluation criteria, beating Assort Health (8/10), partly because the clinical staff found it "much more easier to navigate and easier

		Assort's integration model is more "plug-and-play" (spreadsheet-based), whereas Hello Patient's allows for more complex customization and deeper integration.	seeing real value around the 10th, 11th, or 12th month. The first six months are spent on configuring, optimizing, and gaining confidence in the system.	to work with" operationally. Switching Cost: The system is "sticky" once implemented; switching off Hello Patient was rated 9/10 on a difficulty scale.
Fast (~4 Months Staged)	Found (Telehealth / Weight Loss)	Timeline: Started working with them at the end of last year (late 2024); went live around January 1st. The full implementation, including setup and A/B testing, took approximately a quarter. Involvement: Required light effort from the internal team, mostly uploading content and compliance rules (e.g., specific answers the agent "Mia" must give regarding medications). Hello Patient was building as they flew, suggesting an early-stage partnership.	Phasing: Deployment was staged. They deliberately held back going fully live for about a month to avoid introducing risk during the high-volume Christmas/New Year period, preferring to A/B test against their standard CRM first. Status: Currently live for outbound cart abandon remarketing.	Positive: The implementation required "light effort". The agent has been more effective than their regular CRM for conversion. Challenges: The expert noted that performance reporting and A/B testing tools are still manual and require the Hello Patient team's assistance.
Evaluation Loss (90-Day Pilot)	Shared Practices Group (DSO)	Timeline: Vetting process began Q2 2024. Involvement: The customer asked both Hello Patient and Assort to build specific features/capabilities tailored to their needs. Assort was willing to build custom features (like a phone lookup system), but Hello Patient was not willing to accommodate special customizations, stating, "No, our product's our product".	Pilot Term: Offered a 6-month pilot contract, which was rejected in favor of Assort Health's shorter, preferred 90-day pilot.	Negative (Sales Process): Hello Patient lost the deal despite comparable AI quality because their sales process was "Here's our product, here's what it does," and they failed to be consultative or willing to accommodate specific triage needs.

Competitors NPS breakdown

Assort Health

NPS/Rating (Scale of 1–10)	Customer Likes (Strengths)	Customer Dislikes (Weaknesses)	Desired Additional Capabilities	Competitive Losses & Winner
	• EHR Integration: "Best partnership with	• Horizontal Weakness: Deemed	• Payments: Ability to take	• Flip AI (Winner): Lost the TrustCare

	Athenahealth"; integration is "really easy and quick". • Customization: Willing to build crucial custom features like phone number lookups for patient triage. • ROI/Volume: Quickly reduced missed calls (from >30%) and enabled staff repositioning. • Specialty Depth: Understands "a lot of algorithms" necessary for specialty practices like Retina and Orthopedics.	"very niche" and rule-based; struggled with general, confused, or "intentless" conversations. • Pricing Rigidity: "Didn't budge" on price when challenged by a competitor who was 40% cheaper. • Aggressive Sales: Co-founder's "persistence" and sales style "turned off" at least one customer . • Onboarding: DSO reported initial onboarding "sucked" due to lack of customer success team.	payments over the phone. • Outbound RCM: Development of outbound bill collection. • Referral Platform: Take responsibility for the whole new patient referral platform (including faxes and web forms).	Health deal because Flip offered more functionality (billing resolution) for the price and was more flexible on terms. • Hello Patient (Winner): Lost a deal primarily due to Hello Patient having better scheduling logic and more robust analytics. • Dentina.Ai (Threat): Faces pilot threat due to being 40% cheaper.

Hello Patient

NPS/Rating (Scale of 1–10)	Customer Likes (Strengths)	Customer Dislikes (Weaknesses)	Desired Additional Capabilities	Competitive Losses & Winner
• 9/10 (CortiCare RHC)	• Scheduling Depth: Excels at end-to-end appointment management and benefits verification. • Analytics: Has "pretty robust" reporting and analytics—a key differentiator over Assort. • Customization: Offers customization, though more complex than Assort's method. • Outbound Marketing: Agent was more effective than the regular	• Sales Rigidity: Lost a DSO deal by being rigid: "Here's our product, here's what it does," and unwilling to build custom features. • Integration Complexity: Integration is "a little bit more complex" than Assort's defined, granular process.	• Provider-to-Payer RCM: Focus on Prior Authorizations and Benefits Verification. • Seamless Experience: Needs built-in A/B testing/performance reporting for growth managers. • Support: Expansion into phone support or live chat.	• Assort Health: Lost the DSO deal (Shared Practices Group) because Assort was willing to build custom features and allowed a shorter pilot contract.

	CRM for conversion (Telehealth).			

Hyro

NPS/Rating (Scale of 1–10)	Customer Likes (Strengths)	Customer Dislikes (Weaknesses)	Desired Additional Capabilities	Competitive Losses & Winner
• "Very good" (Analytics tool)	• Content Scraping: Uses a semantic approach to scrape website content for tailored responses. • Enterprise Readiness: Strong integration with telephony systems (Genesys) and high scalability. • Capacity Gained: ROI measured by capacity gained for contact centre agents (not eliminated positions).	• Voice Scheduling Blocked: Full voice scheduling is stalled because Epic's APIs for physician schedules are closed. • Low Value/Bugs: Chatbot delivered low scheduling volume (less than 5%) and required manual QA to check for errors/hallucinations. • Conversational Limitations: Struggles with "extraneous information" or rambling.	• Full Voice Scheduling: Needs to overcome EMR limitations to enable end-to-end voice scheduling. • Call Tracking: AI component to summarize call transcripts to reduce manual QA time. • Outreach: Expansion into outbound call workflows.	• Parakeet Health (Indirect Loss): Lost a potential IDN deal because Parakeet insisted on a per-user/per-agent fee instead of the flat admin fee preferred by the client.

Elise AI

NPS/Rating (Scale of 1–10)	Customer Likes (Strengths)	Customer Dislikes (Weaknesses)	Desired Additional Capabilities	Competitive Losses & Winner
• Partner: 9/10 (Dermatology) • Product Effectiveness: 6/10 (Dermatology) • Ranking: 9/10 to 10/10 (Pre-delay ranking)	• Voice Quality/Data Quality: Voice sounds "like a human being". • Data Integrity: Valued for being "thoughtful and slow" to verify data (e.g., insurance IDs), improving data quality over rushed human staff. • High-Bar Focus: Committed to solving complex new patient intake (triage by insurance/gender/specialty).	• Extreme Delays: Suffered ~18 months of delay from the original go-live target (May 2024). •Bugs/Mistakes: "Kept making mistakes" on complex new patient flows (e.g., pronunciation, triage).	• RCM/Billing: Needs to tackle billing and claims and prior authorizations. • After-Hours Service: Needs to prove it is a better value than the existing after-hours service. • Outbound Re-engagement: Tackle conversations with cancellations/no-shows.	• No specific loss to a competitor disclosed. The customer delayed full rollout due to the product's own technical instability.

		• Manual Customization: Requires heavy reliance on providing Excel documents for workflows and phonetic spellings.		

Infinitus

NPS/Rating (Scale of 1–10)	Customer Likes (Strengths)	Customer Dislikes (Weaknesses)	Desired Additional Capabilities	Competitive Losses & Winner
• Not Disclosed (Evaluator)	• RCM/Payer Specialization: Strong focus on Prior Authorization and Benefits Verification. • Cost Efficiency: AI sitting on hold for 20–50 minutes costs "really no cost" (vs. human FTE). • Data Accuracy: Collects data to correct human agent errors on the payer side.	• High Cost Perception: Considered "really high" cost when pairing it with Assort. • Conversational Flow: AI forces human agents to answer questions, disrupting the typical "data dump" workflow.	• Integration (Vendor Agnostic): Must be able to integrate with multiple scheduling systems due to joint venture needs. • Efficiency Refinement: Must constantly refine the tool to cut out complex, unnecessary steps.	• None Disclosed: The customer (University Radiology) had not adopted any platform at the time of the interview.

DeVC IC Views

https://www.notion.so/devcmain/Confido-Health-12329bccb6fc8059a08fed398744fd5e?pvs=4

1. **Aakrit — Overall: Strong Maybe**

    - We should **not lead** the round.
    - Founders are good and have **figured something out**.
    - Remembers the BurnCal days; **founders have compounded** since.
    - "**More to this traction than meets the eye**" — Cash should get closer to assess.
    - Doesn't see **booked vs live revenue** as an existential issue; founders need some education here.
    - Outcome could be **huge if they get to a few million $ revenue**.
    - Suggests digging deeper into **Curis relationship** — how/why it happened, any backchannel?

- Comfortable with **usage-based revenue** — comparable to workforce deployment in agentic models.
- Wants to see **predictability** via minimum billable contracts and early land-grab.
- Founders have the **right GTM compass** (e.g., counting only first go-lives as pipeline, not inflated by resellers).
- Prefers we **pre-empt and not wait** for a round to form.
- Suggests a **pure convertible** with 20–25% discount over 6 months, since we're not leading.

2. **Vikram — Strong Maybe**

- This is **as good as it gets** at this stage.
- Founders show **strong intrinsic qualities** — outside-in mindset, good seller, balanced on product and GTM.
- Appreciates founder's **thoughtfulness and high-growth mindset**, despite not being a native AI SaaS founder.
- Founder has thought through **multiple GTMs and business models**.
- Positive on the **early scale and product feedback** loops it provides.
- Sees **risk in Curis-based GTM** — it's potentially non-scalable, and future revenue should be **discounted**.
- Valid concern from Ashwini and RA about **booked ARR**.
- Recommends **Cash dig into revenue recognition** process — possible external nudge from Together.

3. **Ashwini — 100% Yes**

- Fully committed to backing the company — "**not even a Strong Maybe**."
- As long as the GTM can be **corrected at scale**, this is investable.
- Sees **revenue recognition issues** as fixable — sales-led accounting interpretation is common.
- Wants to **reset founder compass** on how revenue is tracked and reported.
- Flags **partner-heavy GTM** as potentially risky and binary in outcome.
    - In healthcare, partners may jump to the highest bidder.
    - Must **double down on direct GTM** soon to avoid trap.
- Sees a **UiPath-like partner playbook** as a path to win — if executed with scale and speed.

4. **Nitin — Strong Maybe**

- Sees clear **PMF and land grab opportunity** — 1-week sales cycle is compelling.
- Believes **early advantage is crucial** — especially **if Curis is real and sustainable**.

5. **RA — Positive Bias Toward Commitment**

- Open to committing despite not having **customer references** yet.
- Believes external references are desirable but **not essential**.
- Recommends **getting more outside-in views** before final decision.

- Wants to **validate founder strength and remove intrinsic doubts**.
- Excited about the company and calls it one of the **better ICs done**.

6. **Aakash — Strong Maybe**

- Scores:
  - Spikey: 1
  - FMF: 6–7
  - Thesis: 8–9
  - Archetype: 8–9
  - EF: 7–8
  - Traction: 8
- Believes company is **playing directly to AI-agent labour replacement** trend.
- Founders have **compounded since the last venture**, consistently beating plan.
- Praises **smart GTM strategy** and India cost advantage in solutioning.
- Accepts **usage-based + booked revenue model** as normal in this category.
- Recommends building **predictability with minimum billable contracts**.
- Confident in founder discipline around **pipeline and reseller exclusions**.
- Strong view that we should **pre-empt and invest with early conviction**.
- Suggests **convertible with 20–25% discount** over 6 months given we're not leading.

MPI views

VV views

26 Jan'26

- **Overall View**
  - Conviction in Confido has continued to improve, driven primarily by Chetan's growth as a founder and the team's steady execution in a land-grab healthcare market.
  - This is not a raw, spike-driven bet. It is a methodical founder + execution bet, where the team is making fewer mistakes than peers and building credibility across customers, partners, and investors.
  - Leaning toward adding capital, but only with the right structure. The valuation gap remains meaningful.
- **Founder Read**
  - + Growth mindset - Chetan keeps growing on me meeting to meeting.
  - The steepest growth was visible till Sept–Oct, particularly in judgment and confidence.
  - The last couple of months have been flatter, mainly on org building and live ARR conversion, but not alarming in context.
- **Founder Archetype**
  - Compared to Coral's higher raw spikiness and hustle, Chetan + Vichar come across as:
    - More methodical
    - More intuitive about enterprise GTM
    - Better at org and decision hygiene
    - Making fewer obvious mistakes
  - They are FS-ish operators who've sold before, and that experience shows up in calibration and confidence.
  - Not a 10/10 spike, but ~1.0 Zpiky with steadiness, which matters in healthcare.
- **External Signal & Word of Mouth**
  - Strong ecosystem feedback from Manav @ Memora, with a comparative view across competitors.
  - Positive ambient feedback from PE investors (Gryphon).
  - Customers and partners reinforce feedback
  - Win rates are strong and moved from SMb - ENT
  - Overall, founder credibility appears to be compounding faster than product differentiation.
- **Contracted vs Live ARR**
  - The CARR → live ARR gap is endemic to healthcare.
  - Six-month implementations, workflow redesign, and FD-led selling make this normal. - feature of the model, not a bug.
  - I pushed on slowing down; Chetan's response was to prioritize land-grab and let live ARR catch up. - Given category context, this stance is reasonable.

- **Org & Execution**
    - Org execution is reasonable but not top-decile.
    - There is visible thoughtfulness in the pod-based + FTE / associate structure.
    - Still early, but shows iteration and learning rather than static org design.
    - Reminds me of early Razorpay-like thinking — not in outcome, but in how systems are being formed and adapted.
- **GTM & Hustle**
    - Team is highly visible: JPM, Hawaii, multiple healthcare events.
    - Hustle is supplemented with performance marketing.
    - This explains the consistent pipeline generation.
    - To reach $15M+, they will need:
        - More structured outbound
        - Better resourcing
        - Less dependence on founder-led motion
    - This is a known gap.
- **Valuation & Structure**
    - There is a clear valuation gap.
    - Chetan wants us to underwrite a full forward-priced $30–40M round.
    - That does not feel underwriteable for us today.
    - The only viable path is a convertible / discounted structure, allowing us to lean in while managing price risk.

11 Nov '25

We should get to an answer on Pre-empting this and what price
+ Strong growth trajectory signals
+ Deep thought process on org development. Articulation and point of view on the FDE model resonates well
+ Find him more mature than coral and more proof points on execution (also function of time in market)
? Need to get comfort on quality of revenue and live to contracted risk.

2 May'25

Overall: "Strong maybe" – "Just keeps growing on me; strong maybe even on the DeVC IC

- "IF traction deal" – "We have to underwrite it as an IF traction deal," so the familiar three tests—trajectory, thoughtfulness, and business/org-building—apply.
- "Every time you meet" – "I've met him now three times… he has continuously grown on me," so the trajectory box is emphatically ticked.
- "Thoughtfulness… pre-AI vs. post-AI" – Shows "the thoughtfulness he has both in terms of questions… pre-AI vs. post-AI clarity, voice is a clear wedge, distribution-first thinking," and "reminds me a lot of Harshil in the old days, don't want to jinx it"

- “He loves hiring” – is thinking about all these different types of hires, thinking about channel partnerships very early,” proving he already thinks about business- and org-building.
- “Pipeline velocity and product word-of-mouth” – Early signals on “pipeline velocity and product word-of-mouth… were there,” yet Pranay warns the product feels “clunky,” so we need to reconcile that gap.
- “Be clear this is amongst the best products” – We must confirm it really is “amongst the best products in the market.” Acknowledged risk: “there is not a 10X differentiation because it is actually voice and it is going to get commoditized.”
- “Product demo and tech deep dive” – I’ve “opened the door for that meeting”… spend tomorrow morning with him on usage data, do a full demo + tech dive, pull cohort metrics, and align on the 4-5 key product-usage questions over email before the call.
- “Horizontal strategy over a period of time” – Leap of faith is that his current horizontal strategy evolves to build vertical moats and a “sizable business (call it $70-100 M).”
- “Infinity founder… not that absolutely today” – Bet that over a period of time he tries to become a infinity founder because he's not that right now.
- “$3 M, 10 % in the company” – with 1M together and close fast before it drifts to a $10 M raise—“I’m bullish if we get in on those terms.”

Refer DeVC notes as well for VV’s older views

<u>KP Views</u>

26 Jan’26

<u>Overall a SMB</u>

+ Chetan has delivered on an aggressive target and increasingly closing large enterprise deals
+ The PE playbook was what I had the least confidence on last time and he’s proving that he can make it work
+ Hiring a top class team; Raihan is well reputed and great to see him pull talent like this
+ If he keeps up the $1M/ month this will be on a rocket ship trajectory
+ Chetan constantly growing on me. Also seeing him be more confident that he can win in this space
? Contracted to live has been slow. But it’s still at the same 2 month lag time that he mentioned. Growth on ARR is much faster than he can hire
? Risk of missing the target incase he isnt able to crack more PEs. This is a tough channel.
? While team buildout on business has been good, engineering needs to be validated

11 Nov’25

+ Compounding and ahead of targets
+ Strong traction and enterprise motion kicking in; $2.8M+ and will hit $4M by EOY
+ Thoughtful, mature

+ early signs of repeatable GTM showing up
? Share the same risk on Live to contracted lag
? Need a view on pace and quality of org buildout

2 May'25

Strong maybe. First meeting was a WMB but as I spent more time I have come to like him a lot more. Hearing a lot of positive signals around him and what he says is being checked out by experts and customers.

- Spiky founder – very strong on intrinsics; Cant pinpoint spiky on GTM/ product but given its an FF, our read should be oriented accordingly
- Product is at par with competition in qualitiy; not a space where you can epect to see 10x difference. Voice AI will sounds similar given they're all building of similar tech stack
    - I had to force negative things out of customers when we spoke

1. Very iterative on GTM and trying all the right things. Partnerhsip and events are a channel we arent used to seeing in SaaS this early, but almost eveyone in healthcare relies on these and experts we spoke to validated his approach
    - Early signs of the partnership channel working. Wether he can repeat it is a leap of faith

- What he says + our work are tying in well
- Market oppurtunity is huge and Is here and now
- We should validate product usage metrics

17 April

Overall WMB but want to spend more time on this. Founder seems sharp and scrappy and like the traction over the last few months, especially since he met plans. Ɨm still unclear if this is scalable traction and would spend time there
+ High IQ, pedigree
+ Hustle – lived on one meal a day to make this work
+ Thesis and TAM – large market and open pain point; caveat here is that any company can scale and get to a 5-6M scale
+ Traction – discounting the live vs contracted; he's gotten to 1M and something is working so should understand this deeper.
Revenue ramp for older customers + feedback from some of the newer ones
? Risk on GTM (Partnerships) and revenue converting to live
+/? Product – we should dig deeper to understand the technical differentiation and nuances
+/? Team buildout – good not great; Recent head of AI profile looks quite strong though
? Sharpness on ICP missing
? Hustle over thoughtful

<u>PD views</u>

2 May

Maybe -ve

See more risk than reward
Spiky rating of 0.5, unsure what they hard capability spike is
Is the product truly best in class

Looked like an old world IVR workflow setup
But then again we don't know if competitors are much better
Calibrating on best in class voice AI key

Is there any impact with the CTO leaving?

TAM not an issue and this is one of those massive TAM markets

24 Dec

Overall: was a WMB for the fund, and more a question on the founder and strategic choices. Would push on some of the below to build conviction that this can be a big outcome

+ Bullish about the thesis overall, but prefer if the founder can do enterprise GTM and sales

+ Momentum – good not great

? GTM choices – selling through distributors early = no high velocity sales playbook, and 1 level away from product feedback so don't like it

? Don't see repeatability of persona + pain point + product solution. Seems like each customer is different

? Small ACVs – from a customer set who can pay a lot more

? clarity of thinking on segments – who to go after vs. who not to chase. Eg: another founder said – only secondary care – get referrals from primary care via fax + painful paper process

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Appendix

Additional links

1) MPI eval of Confido output and Assort output **+14692849361**
2) Confido product recording
3) Confido product demo